



EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°086/MVEBDB/08/08/2026

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HIGHLIGHTS (last 24 hours)

In the last 24 hours, 85 new confirmed cases (29 new deaths) of Ebola Virus Disease (EVD) cases were recorded in the provinces of Ituri (68 cases), North Kivu (10 cases) and Haut-Uélé (7 cases), reflecting the continuation of active transmission, mainly in Ituri.

CUMUL CASES 4,294	ACCUMULATIVE DEATH CONFIRMED** 1960	LETHALITY 45.6%	PATIENTS IN ISOLATION / CTE 722	ACCUMULATIVE HEALED 849	TRACKING RATE CONTACTS (Of the Day) 84.6%
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Affected Provinces and Affected Health Zones

5 Provinces Affected

Haut-Uélé, Ituri, North Kivu,
South Kivu & Tshopo



53 Health Zones
affected Out of 140
(37.8%)



246 Health Areas
touched
Out of 3104 (7.9%)

Summary of key points from the last 24 hours

- In the past 24 hours, 85 new confirmed cases (29 new deaths) of Ebola Virus Disease (EVD) have been recorded in the provinces of Ituri (68 cases), North Kivu (10 cases) and Haut-Uélé (7 cases), reflecting the continuation of active transmission, mainly in Ituri.
- To date, the Democratic Republic of Congo has recorded 4,294 confirmed cases, including 1,960 deaths, representing a case fatality rate of 45.6%. Ituri remains the epicenter of the epidemic, accounting for 86.4% of cumulative confirmed cases and 82.0% of new cases reported in the last 24 hours.
- 21 patients were declared cured in Ituri after obtaining two negative results for Ebola virus disease and 15 intra-CTE deaths recorded in Ituri and North Kivu.
- The epidemic affects 53 of the 140 health zones in the five affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (6/23) and South Kivu (1/34), confirming the geographical spread of the disease.

Distribution of confirmed cases and deaths by affected province

(as of August 8, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	3,704	1,584	42.8% 28/36	(77.8%) 67.9% 12/34	68
North Kivu	480	326	(35.3%) 44.9%	6/13 (46.1%)	10
Haut-Uélé	98	44	55.6% 6/23	(26.0%) 1/34 (2.9%)	7
Tshopo	9	5			0
South Kivu	3	1	33.3%		0
Total	4,294	1960	45.6%	53/140 (37.8%)	85

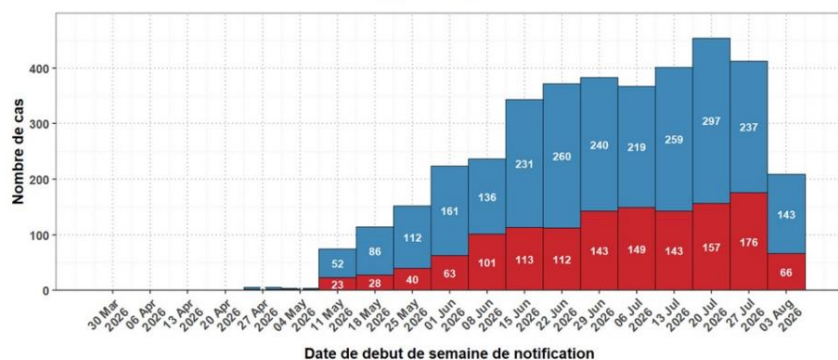
Status of alerts notified by ZS, as of August 8, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (as of today)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	4	1	5	4	1	0	0	5	5
Ituri	500	77	577	108	77	392	0	185	83
North Kivu	631	19	650	45	19	586	0	64	34
South Kivu	7	0	7	2	0	5	0	2	0
Tshopo	43	0	43	2	0	31	0	2	2
Total General	1,185	97	1,282	161	97	1014	0	258	124

Evolution of cases by date of symptom onset over the last 21 days

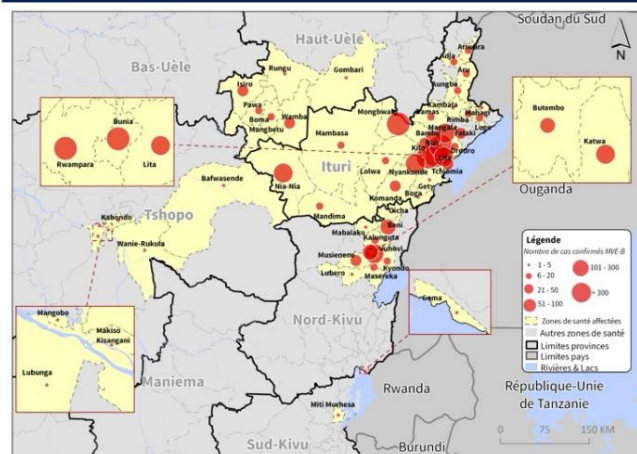
Cas confirmés vivants et décédés par semaine de notification (n = 3 757)

■ Décès ■ Vivant



Source : DHIS2 Tracker - Riposte MVE, août 2026. Données incluses : cas confirmés avec statut vital et date de notification renseignés, n = 3 757.

MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : Joint WHO-DPS mission in the Vuhovi health zone, for the assessment of the functionality of the zonal coordination of the response.

1.1.2. Challenges

- Furthermore, several strategic actions essential to the decentralization of coordination remain pending, notably the establishment and operationalization of the sub-coordinations in Wamba, Pawa, and Watsa. This delay reduces coordination capacity at the operational level and necessitates accelerating the administrative, logistical, and organizational measures required to make these sub-coordinations fully functional.

1.2. Epidemiological surveillance

1.2.1. Main Actions

1,282 alerts were recorded and verified in the five affected provinces on August 8, 2026, of which 258 (20.1%) were validated as suspected cases and 258 (100%) were investigated and 124 suspects transferred to CT/CTE. Contact tracing is at 84.6% (14,505 seen out of 17,141 to be followed), corresponding to the threshold of 85%, with low performance in Haut-Uélé (76.1%) and Nord-Kivu (80.2%); Ituri reaches 87.7%.

1.2.2. Challenges

Low reporting of alerts and poor contact tracing around confirmed cases

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

Nine alerts were detected at the PoE/PoC (7 in Ituri, 2 in Haut-Uélé): the five live alerts in Ituri were validated, isolated and referred to the care structures; the two lifeless bodies intercepted in Ariwara were not collected, due to the lack of an available EDS team, and none of those in Haut-Uélé were validated.

135,404 travellers were checked (117,337 in Ituri, 18,067 in Haut-Uélé), with a screening rate of 97.4%. /Strategic PoCs out of 18 (5.6%) operated without interruption, compared to 100%.

1.3.2. Challenges

• Insufficient operationalization of PoE/PoC: PoE and PoC functionality remains low in Haut-Uélé, with only 10 out of 38 (26.3%) operational. • Constraints affecting PoE/PoC functionality: Performance continues to be affected by shortages of supplies, PPE, and water in Haut-Uélé, non-payment of service providers, and non-compliance with health protocols by some authorities at the Mukulia PoC (North Kivu). These constraints reduce the effectiveness of health control, early detection, and surveillance at borders and along major transportation routes.

1.4. Laboratory

1.4.1. Main actions

- **400 samples** were analyzed, confirming **85 new cases** of EVD (positivity: **21.3 %**).
- Ituri recorded **68** positive results, including 24 deaths, while North Kivu and Haut-Uélé confirmed **10 (5 deaths) and 7 cases respectively**.

1.4.2. Challenges

High proportion of positive samples in Ituri (26.0%) and incomplete reporting of laboratory results in Haut-Uélé and Tshopo (no data transmitted), limiting biological surveillance and real-time monitoring of Ebola transmission.

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- In **Ituri**, 25 rings were opened (92 in total), 85 sites identified and 39 decontaminated (45%); 66 EDS carried out on 82 death alerts (80.5%) and 50 bodies collected post-mortem, despite a completeness limited to 10 ZS out of 29 (34%).
- In **North Kivu**, 76 health facilities were supported and 229 providers were briefed; 7 case stay facilities were decontaminated, 36 EDS were carried out and 37 bodies were collected and secured, for a completeness of 4 out of 12 health zones. • Tshopo did not transmit any IPC/EDS data.

1.5.2. Challenges

Poor implementation of IPC standards: very low reporting completeness in Ituri (34%) and North Kivu (4 out of 12 health zones), partial decontamination of identified sites in Ituri (39 out of 85) and low card scores in North Kivu (30.3% in Beni and 31.4% in Kalunguta), demonstrating a continued risk of healthcare-associated infections.

1.6. Continuity of care

1.6.1. Main actions

- **In Ituri**, 29 ambulances were assigned to the response, of which 26 were available; 28 trips were made, 26 of which were successful, for 29 evacuated patients. The Ebola Treatment Centers (ETCs) recorded 73 admissions and 82 discharges, of which 21 recovered; occupancy reached 501 patients out of 840 beds (60%), with the ETCs in Rwampara, ISTM, and Nizi, and the Bambu Treatment Center, at full capacity.
- **In North Kivu**, there have been 239 cumulative admissions to CTE/CT facilities, including 208 confirmed cases, with an intra-CTE mortality rate of 39%; 66 admissions, no recoveries today while 6 intra-CTE deaths have been reported.
- **In Haut-Uélé** : the absence of standardized CTE persists in the 6 affected health zones

1.6.2. Challenges

Insufficient availability of referral resources (3 ambulances out of service in Ituri, no ambulance in 7 health zones in North Kivu) and saturation of several CTE/CT facilities, limiting rapid access to care and isolation of suspects.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, mass awareness-raising was conducted among 100 vendors at the Sukisa 2 market, and 20 community health workers were accompanied during door-to-door visits in the Rwampara health zone; 4,213 people were reached during the home visits.
- **In North Kivu**, 17 community leaders were briefed and a guided tour of the Kitatumba CTE organized; 16 rumors were clarified, 5 community incidents resolved and 129 household heads affected around 25 cases.
- **In Haut-Uélé**, 2,332 people were reached during visits to 749 households in Isiro, Wamba and Pawa, 30 motorcycle taxi drivers were sensitized in Wamba and 66 IEC tools were distributed (cumulative: 351).

1.7.2. Challenges

Persistent community challenges: demotivation of stakeholders due to non-payment for over two months in North Kivu, denial of Ebola virus disease and opposition to a disease prevention and control program in Isiro, repeated delays in vaccine availability, likely to affect community participation in response interventions

1.8. Logistics

1.8.1. Main actions

In Ituri : the installed capacity reaches 840 beds (412 for confirmed cases, 428 for suspected cases); the extension of the CTEs in Bambu, Nizi and Logo is requested in the face of saturation.

In North Kivu : continued unpacking of the WHO donation of PCI kits, preparation of orders for the Vuhovi and Musienene health zones and delivery of infant milk to the Butembo health zone; work in progress for the bases of the tanks at the PoCs of Karuruma and Komba and for the triage unit of the Vuhovi General Referral Hospital (ULB Cooperation).

1.8.2. Challenges

Persistent logistical needs to support the expansion of the response, including the completion of CTE extensions, the continued supply of essential inputs and PPE, and the availability of means of transport and evacuation

1.9. Security

1.9.1. Main actions

- In North Kivu, 13 security escorts were provided to EDS and PCI teams in Lubero and Katwa; intervention by the security team in Katwa, with warning shots, in response to an attempt by the population to recover the body of a confirmed case.

1.9.2. Challenges

- Persistent insecurity, incidents involving political authorities and community resistance during safe and dignified burials, limiting access for response teams, continuity of interventions and coverage of activities in some affected areas: attempted recovery of the body of a case confirmed by the population in Katwa (North Kivu) and 15 safe and dignified burials postponed until the following day in Ituri.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

Continued awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA) in the various communities of the affected provinces.

1.10.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse remains insufficient across all areas covered by the response.

KEY MESSAGES

- Transmission of Ebola virus disease remains active, with 85 New cases and 29 confirmed deaths recorded in the last 24 hours, bringing the national total to 4,294 confirmed cases and 1,960 deaths.
- Ituri remains the epicenter of the epidemic, accounting for 86.4% of cumulative confirmed cases and 82.0% of new cases reported in the last 24 hours, with a more significant expansion (28 of the 36 health zones affected). • Surveillance is performing well in the last 24 hours, with 100% of alerts investigated and a follow-up rate of 84.6%, corresponding to the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic went from detection of the first suspected cases through confirmation biological of the Ebola Bundibugyo virus, leading to the official declaration of the 17th epidemic of Disease Ebola virus in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, the national authorities, with the support of partners, strengthened the response by the declaration of a public health emergency of national and then continental scope, accompanied by the mobilization of coordination mechanisms and support.



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